

Department of Pediatric Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 5.9 Yr/M
Admission No.: anonymous	Department Unit: Pediatric Surgery (Ped Sx Unit 1)	Consultant/Faculty: anonymous
Ward: Pedia Sx 3 C 4	Room/Bed: 3 C 4 Ward/ 3 C 4 Pae 25	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 25-Mar-2023 18:11	D.O.D. : 08-Feb-2026	D.O. Print Report: 08-Feb-2026 15:30

FINAL DIAGNOSIS:

EMPYEMA LEFT CHEST

EMPYEMA LEFT CHEST

CHIEF COMPLAINTS:

K/C/O COMMUNITY ACQUIRED PNEUMONIA WITH LEFT EMPYEMA WITH ICD IN SITU TRANSFERRED FROM PEDIATRIC MEDICINE DEPARTMENT

HISTORY OF PRESENT ILLNESS: : ACCORDING TO PATIENT ATTENDER, 15 DAYS BACK PATIENT DEVELOPED COUGH WHICH WAS ACUTE IN ONSET AND GRADUALLY PROGRESSIVE, A/W POST TUSSIVE VOMITING, AND HIGH GRADE FEVER (NON DOCUMENTED). PATIENT WAS ADMITTED FOR THE SAME UNDER PAEDIATRICS AND EVALUATED. HE WAS DIAGNOSED TO HAVE LEFT EMPYEMA AND HENCE WAS TRANSFERRED TO PAEDIATRIC SURGERY FOR FURTHER MANAGEMENT.

EXAMINATION FINDINGS :

GENERAL:

PR: 114/M TEMP: 98.2F

RR: 24/MWEIGHT : 8.9 KG

No Pallor, No Cyanosis, No Clubbing, No Generalised LNE, No Generalised Edema

CNS: WNL

CVS: S1S2 HEARD

RS: DECREASED AIR ENTRY ON LEFT SIDE, ICD IN SITU ON LEFT SIDE, PUS COMING FROM ICD

PA: SOFT, NO TENDERNESS

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
25-Mar-2023 19:44	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.23	0.02 - 0.5 x10 ³ /μL

	Absolute Lymphocyte Count	2.61	1.0 - 3.0 x10 ³ /μL
	Absolute Monocyte Count	0.90	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	4.36	2.0 - 7.0 x10 ³ /μL
	Basophil	0.1	00 - 01 %
	Eosinophils	2.8	01 - 06 %
	HCT	34.3	34 - 40 %
	Hemoglobin	10.0	11 - 14 g/dl
	Lymphocytes	32.2	44 - 74 %
	MCH	23.8	24 - 30 pg
	MCHC	29.2	31 - 37 g/dL
	MCV	81.5	83.0 - 101.0 fL
	Monocytes	11.1	02 - 10 %
	Neutrophils	53.8	40 - 80 %
	Platelet Count	476	150 - 410 x10 ³ /μL
	RBC	4.21	4.5 - 5.5 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	20.1	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	8.11	4.0 - 10.0 x10 ³ /μL
HBsAg (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
HCV Antibody (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
Rft	Chloride	103	102 - 109 mmol/L
	Creatinine	0.40	0.9 - 1.3 mg/dl
	Potassium	4.09	3.5 - 5 mmol/L
	Sodium	137	135 - 145 mmol/L
	Urea	21	15.0 - 39.0 mg/dl

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: CECT: LOCULATED COLLECTION IN POSTERIOR LOBE OF LEFT LUNG

HOSPITAL COURSE: PATIENT IS A K/C/O COMMUNITY ACQUIRED PNEUMONIA WITH LEFT EMBYMA WITH ICD IN SITU. HE WAS MANAGED CONSERVATIVELY WITH ORAL AND IV ANTIBIOTICS AND INCENTIVE SPIROMETRY. ICD WAS REMOVED AFTER COLLECTION REDUCED AND LEFT LUNG EXPANSION WAS SEEN ON CXRAY. POST ICD REMOVAL XRAY WAS NORMAL, PATIENT SYMPTOMATICALLY IMPROVED, HENCE BEING DISCHARGED WITH FOLLOWING ADVICE.

CONDITION AT DISCHARGE: : PATIENT IS HEMODYNAMICALLY STABLE. PASSED URINE STOOLS. NO COUGH OR SHORTNESS OF BREATH

ADVICE ON DISCHARGE:

CONTINUE CHESTPHYSIOTHERAPY

INTENSIVE SPIROMETRY

SYRUP MVBC 2.5 ML OD X 15 DAYS

HIGH PROTEIN DIET



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



REVIEW AFTER 1 WEEK

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 10-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW WITH CHEST X RAY

Summary Prepared By

anonymous

Summary Approved By

anonymous